ClaimGuard Sample Coverage Policy: MRI of the Lumbar Spine

SYNTHETIC POLICY FOR TESTING. Written for the ClaimGuard project and modelled on criteria found in public lumbar spine MRI policies. Not the policy of any real health plan.

Policy number: CG-SAMPLE-001. Applies to CPT codes 72148, 72149 and 72158.

1 Purpose

This policy describes when magnetic resonance imaging (MRI) of the lumbar spine is considered medically necessary for adults with low back pain or lumbar radiculopathy. Most episodes of acute low back pain improve within six weeks with conservative care, and early imaging without a clear indication does not improve outcomes. Prior authorization is required before the study is performed.

2 Covered Indications

MRI of the lumbar spine is considered medically necessary when the request meets at least one of the indications in Section 2.1, Section 2.2 or Section 2.3, and the documentation requirements in Section 4 are satisfied.

2.1 Red Flag Conditions

Imaging is considered medically necessary without a trial of conservative therapy when the clinical notes document any of the following red flag conditions: symptoms of cauda equina syndrome, such as new urinary retention, bowel incontinence or saddle anesthesia; a progressive or severe neurological deficit, such as worsening foot drop or rapidly increasing weakness; suspected spinal infection, such as fever with back pain in a patient with intravenous drug use, recent spinal procedure or immunosuppression; suspected malignancy, such as a history of cancer with new back pain or unexplained weight loss; or suspected vertebral fracture after significant trauma or in a patient with osteoporosis or long-term steroid use. When a red flag condition is documented, the duration of symptoms is not a requirement.

2.2 Persistent Symptoms After Conservative Therapy

In the absence of red flag conditions, imaging is considered medically necessary when both of the following are documented. First, low back pain or radicular symptoms have persisted for at least six weeks. Second, the patient has completed at least six weeks of physician-directed conservative therapy that did not adequately relieve symptoms. Conservative therapy means physical therapy or a supervised home exercise program, and may also include anti-inflammatory medication, activity modification or chiropractic care. Medication alone does not meet the conservative therapy requirement. Requests with symptoms for six weeks or longer but no conservative therapy do not meet this section.

2.3 Surgical or Interventional Planning

Imaging is considered medically necessary when the patient is being evaluated for surgery or an epidural steroid injection, and the requesting provider documents that the result will be used to plan that procedure.

3 Not Medically Necessary

MRI of the lumbar spine is not considered medically necessary for uncomplicated acute low back pain, with or without radiculopathy, that has lasted less than six weeks when no red flag condition from Section 2.1 is documented. MRI is also not considered medically necessary for routine screening, or when the stated reason for imaging is the patient's request alone.

4 Documentation Requirements

The request must include the following information, or it cannot be reviewed and will be returned for additional information. The duration of current symptoms, stated in days, weeks or months. A description of the conservative therapy completed, including the type of therapy and how many weeks it lasted. The findings of a neurological examination, including strength, reflexes and sensation in the lower limbs. The ICD-10 diagnosis code or codes that support the request. The date and result of any previous lumbar spine imaging. A request that is missing any of these items is returned to the provider as incomplete rather than denied.

5 Repeat Imaging

A repeat lumbar spine MRI within twelve months of a previous lumbar spine MRI is not considered medically necessary unless there are new or significantly worsening symptoms or neurological findings since the previous study, a red flag condition from Section 2.1, or planned surgery as described in Section 2.3. Unchanged chronic symptoms are not a reason for repeat imaging.

6 Coding Information

CPT codes covered by this policy: 72148, MRI lumbar spine without contrast; 72149, MRI lumbar spine with contrast; 72158, MRI lumbar spine without contrast followed by contrast.

Examples of supporting ICD-10 diagnosis codes: M54.50, low back pain, unspecified; M54.16, radiculopathy, lumbar region; M54.17, radiculopathy, lumbosacral region; M51.26, other intervertebral disc displacement, lumbar region; M48.061, spinal stenosis, lumbar region without neurogenic claudication; M48.062, spinal stenosis, lumbar region with neurogenic claudication; G83.4, cauda equina syndrome; C79.51, secondary malignant neoplasm of bone; S32.009A, fracture of unspecified lumbar vertebra, initial encounter.

7 Review Process

Requests that meet this policy are approved. Requests that do not meet this policy are reviewed by a physician reviewer before any denial is issued, and the requesting provider may request a peer-to-peer discussion or file an appeal.